

COMMUNITY HEALTH



Community health

Bangladesh's NCD crisis is huge, under-treated, and addressable by community health workers



✓ PROVEN PRECEDENT

COBRA-BPS (NEJM): CHW care works at <\$2/capita/yr

▲ WHY IT MATTERS

NCDs = 67% of deaths · 61.5% of diabetics undiagnosed

THE SITUATION IN BANGLADESH

In Bangladesh, noncommunicable diseases now account for 67% of all deaths, and nearly one in five adults risks dying from an NCD between ages 30 and 70¹. Hypertension affects roughly a quarter of adults, yet only 36.7% are even aware they have it, just 31.1% are treated, and a mere 12.7% have it controlled². Diabetes tells the same story: around 13.1 million adults are affected, placing Bangladesh among the world's top 10 countries for diabetes, with nearly 5.7 million cases undiagnosed⁶, and 61.5% of people with diabetes unaware of their condition⁷. The gap is worst in rural areas, where undiagnosed hypertension and diabetes are widespread and regionally unequal, leaving millions to reach advanced disease before anyone detects it⁵⁹.

OUR PITCH

Nirog closes this detection-and-control gap by pairing trained community health workers with simple AI screening tools, mirroring the model proven in the COBRA-BPS cluster-randomized trial, where CHW-led multicomponent care lowered systolic blood pressure by about 5 mmHg more than usual care across rural Bangladesh, Pakistan and Sri Lanka³. That same trial was highly cost-effective, projected to cost under US\$2 per capita annually to scale, making door-to-door screening and chronic-care follow-up economically viable⁴. Our protocols follow the WHO Package of Essential NCD interventions (PEN), explicitly designed so non-physician health workers can detect and manage cardiovascular disease and diabetes in low-resource primary care⁸. The wedge is an institutional payer that funds screening and ongoing management as a cost-saving investment, because early detection and control of hypertension and diabetes is both clinically effective and cheap relative to the strokes, heart attacks and kidney failure that follow late diagnosis³⁴.

Built so the operator never fails

HelperChain doesn't hand out a loan and walk away. Every operator gets the **full rail** — inputs, training, finance, supervision and a **guaranteed buy-back** of what they produce. We carry the risk so an ordinary person can succeed.



Equipped & trained

We provide the tools, inputs and hands-on training to start — turning **household** into a working operation. No prior capital or expertise required.



Funded, not indebted

Pay-as-you-go, **riba-free** finance turns a poor person into a **funded** operator — with **no debt if a cycle fails**. The network carries the downside, not the operator.



Supervised & supported

Ongoing follow-up and quality assurance — the single factor peer-reviewed evidence shows decides whether ventures survive (**p<0.001**). The operator is never left alone.



Guaranteed buy-back

HelperChain **commits to purchase the output at a fair, pre-agreed price**, so there is always a market — the operator just produces; **we guarantee the offtake**. This is what removes the fear of failure.



Surplus recycles

A thin margin is recycled to fund the next operator — so help compounds and the network grows stronger with every person it lifts.

The promise: we give the operator everything needed to succeed — and we buy back the output at a fair, pre-agreed price. Inputs in, output bought back, risk carried by the network — so they never fail alone.

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